

Digital Rehabilitation and Cybernetic Augmentation for Involuntary Prisoners and Individuals with Severe Mental Illness: Legislative Framework and Model Bill

Part I - Preliminary

Division 1 - Purpose and Scope

This legislative proposal establishes a comprehensive legal system for the governance, deployment, and oversight of **digital rehabilitation and cybernetic augmentation** technologies for involuntary prisoners and individuals with severe mental illness, both in Australia and harmonized with international standards. Its aim is to foster a rights-protective, operationally viable, and ethically robust environment for technology-driven rehabilitation, ensuring alignment with human rights, medical ethics, and the values of inclusion and justice^[2].

Division 2 - Definitions

For the purposes of this Act, unless the context indicates otherwise:

- **Digital Rehabilitation Technology (DRT):** Any information and communication technology (hardware or software) used to deliver or facilitate therapeutic, educational, behavioural or psychosocial rehabilitation, including tablet computers, virtual reality systems, telepsychiatry, web-based platforms, biometric monitoring, and algorithmic decision-support systems^{[4][5]}.
- **Cybernetic Augmentation (CA):** Any implanted or externally worn prosthetic, neural device or computer interface designed to restore, enhance, or compensate for motor, cognitive, psychiatric, or sensory function in persons with disabilities or health/mental conditions.
- **Involuntary Prisoner:** Any individual detained by state authority under criminal law or security legislation, including sentenced prisoners and persons under forensic or preventive detention.
- **Severe Mental Illness:** As defined in the Mental Health Acts (e.g., schizophrenia, major affective disorders, psychotic disorders, severe PTSD, or dual diagnosis), including persons found not criminally responsible by reason of mental impairment.
- **Informed Consent:** Consent freely given after full disclosure, in accordance with medical, legal, and ethical requirements as per this Act and relevant statutes^[7].
- **Designated Oversight Body:** A statutory agency or committee tasked with monitoring, reviewing, and investigating the use of DRT and CA under this framework.
- **Aboriginal and Torres Strait Islander Prisoners:** In accordance with the definition under the Aboriginal and Torres Strait Islander Peoples Recognition Act 2013 (Cth), with special reference to unique cultural, health, and rehabilitation needs.

Part II - Eligibility, Consent, and Access Protocols

Division 1 - Eligibility and Assessment

Section 1.01 - Assessment Criteria

(1) The use of DRT/CA in involuntary settings shall be **strictly limited to individuals**:

- (a) Who are involuntarily detained, or are legally subject to mandated psychiatric or correctional intervention;
- (b) For whom a multidisciplinary team recommends DRT/CA as a necessary or substantially beneficial means of rehabilitation as compared to less intrusive alternatives;
- (c) Who meet clinical eligibility criteria, as established by evidence-based practice guidelines, differentiated for criminal justice and mental health contexts^[9].

(2) Assessment must consider:

- Rehabilitative needs, risk profile, digital literacy, cognitive capacity, cultural context, and potential vulnerability to coercion or harm.

Section 1.02 - Exclusions

DRT/CA must **not be used**:

- (a) Solely for punitive, disciplinary, or security purposes without primary rehabilitative intent;
 - (b) For the purpose of experimentation, unless following strict ethical and legal research protocols;
 - (c) In persons with mental or physical conditions that would be exacerbated by such intervention, as per Rule 109 of the Nelson Mandela Rules^[10].
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Division 2 - Consent and Decision-Making Safeguards

Section 2.01 - General Principle

(1) The principle of **informed, supported, and culturally safe consent** is paramount.

(2) All efforts must support the individual's autonomy and understanding, in accordance with the Australian Law Reform Commission's recommendations for a supported decision-making model in health and corrections^[11].

Section 2.02 - Capacity Assessment

- Capacity to consent must be assessed for each individual, using established clinical and legal criteria:
 - Ability to understand information relevant to the decision;
 - Ability to retain, use or weigh that information;
 - Ability to communicate the decision (verbally or by other means).

Section 2.03 - Proxy and Substitute Consent

(1) Should an individual lack capacity, consent can be given:

- (a) By a legally-appointed substitute decision-maker (e.g., Guardian), provided this is in the person's best interest or in accordance with their known wishes, and not contrary to established rights regimes^[7];
- (b) By a court, tribunal, or review board if required, subject to regular review and clear justification.

(2) Consent for intrusive or irreversible augmentation (e.g., neural implants) **must be subject to external review** by a Mental Health Review Tribunal, Human Research Ethics Committee, or Court.

Section 2.04 - Withdrawal of Consent

- The individual or their substitute may **withdraw consent at any point**, ceasing the intervention unless continued therapy is deemed vital in a life-saving or health-protective emergency, as defined by health law.
- Appropriate mechanisms for review, appeal, and complaint must exist for all parties, including third-party advocacy and culturally appropriate support^[11].

Part III - Regulatory Framework and Amendments to Existing Laws

Division 1 - Criminal Justice Statutes

Section 3.01 - Amendments to Corrections Legislation

- The use of DRT/CA in prisons shall be expressly authorized and regulated, through new provisions or amendments to Acts such as the Crimes (Administration of Sentences) Act 1999 (NSW), Correctional Services Acts, and equivalent statutes.
- New regulations shall permit:
 - Access to digital devices for rehabilitative and educational purposes under secure protocols;
 - Prohibition of technology use as punishment;
 - Mandatory reporting of adverse outcomes and data collection for evaluation and oversight;
 - Enumeration of prisoner rights in relation to digital therapies^[4];
 - Inclusion of statutory functions for cultural, psychological, and community input in design and delivery.

Section 3.02 - Integration with Criminal Code

- Prohibit use of DRT/CA for cruel, inhuman, degrading, or experimental purposes;

- Align digital interventions with hate crime, discrimination, and privacy provisions;
 - Amend offences to add aggravated penalties for non-consensual implantation or coercive digitisation^[12].
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Division 2 - Mental Health Legislation

Section 3.03 - Mental Health Acts Amendments

- Amend state and territory Mental Health Acts to:
 - Include DRT/CA within the range of “special medical treatments” subject to tribunal oversight;
 - Require documentation of efforts to obtain voluntary and supported consent;
 - Adopt national best-practice standards for the use of digital interventions, as developed by the Australian Commission on Safety and Quality in Healthcare, and the Therapeutic Goods Administration (TGA)^[13];
 - Ensure assessment and approval of digital/augmented therapies by multidisciplinary teams with explicit attention to cultural safety for Aboriginal and Torres Strait Islander people^[15].

Section 3.04 - National Consistency

- Advocate for harmonisation of consent standards and protections across state and territory jurisdictions, modelled on the recommendations of the Australian Law Reform Commission and international best practice^[9].
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Division 3 - Technology, Medical Device, and Privacy Laws

Section 3.05 - Regulation of DRT/CA as Medical Devices

- DRT/CA must be regulated as **medical devices** within the meaning of the Therapeutic Goods Act 1989 (Cth) and associated regulations.
- All devices must:
 - Undergo risk assessment and certification before use (including cybersecurity evaluation)^{[17][18]};
 - Meet Essential Principles and international safety standards (ISO 14971, IEC 62304, IEC 80001)^[16];
 - Be included in the Australian Register of Therapeutic Goods (ARTG) unless exempted by the TGA under special circumstances^[18].
- Software-based mental health and therapy tools must:

- Reference and display clinical guidelines, as per TGA exclusion criteria;
- Undergo voluntary accreditation against the National Safety and Quality Digital Mental Health (NSQDMH) Standards, where appropriate^[13].

Section 3.06 - Privacy and Data Protection

- All data collected by DRT/CA are to be managed in strict compliance with the **Privacy Act 1988 (Cth)**, state privacy laws, and international standards (e.g., GDPR-analogous provisions), including:
 - Confidentiality, minimization, fair and transparent processing of data;
 - Special protections for biometric and neural data, recognizing the unique risks of mental privacy breaches;
 - Right of access, correction, and redressal for prisoners and patients.

Part IV - Safeguards for Human Rights, Privacy, and Medical Ethics

Division 1 - Human Rights Norms and International Law

Section 4.01 - Adherence to UN Standards

- All use of DRT/CA must comply with and give effect to:
 - The **Universal Declaration of Human Rights (UDHR)**;
 - The **International Covenant on Civil and Political Rights (ICCPR)** and the **International Covenant on Economic, Social and Cultural Rights (ICESCR)**;
 - The **UN Standard Minimum Rules for the Treatment of Prisoners (Nelson Mandela Rules)**, particularly as to dignity, non-discrimination, minimum health standards, informed consent, and prohibition of cruel, inhuman or degrading treatment^[11];
 - The **Convention on the Rights of Persons with Disabilities (CRPD)**, especially Article 17 (integrity of the person), Article 25 (health), and Article 15 (freedom from non-consensual medical or scientific experimentation).

Section 4.02 - No Exception for Technology

- Technology must never be used as an excuse or pretext for violating rights otherwise protected by law:
 - No DRT/CA shall be used for torture, coercion, forced confession, indefinite surveillance, datafied “profiling” for punitive purposes, or to augment sentences beyond those judicially imposed^[19].

Division 2 - Medical and Digital Ethics

Section 4.03 - Medical and Digital Ethics Codes

- All professionals involved in recommending or delivering DRT/CA must comply with medical and digital ethics, including:
 - Fundamental principles of autonomy, beneficence, non-maleficence, and justice;
 - Adherence to best-practice clinical guidelines and research standards;
 - Ongoing assessment of risk, side effects, cultural safety, and long-term wellbeing^{[21][22]}.
- Explicit prohibition of coercion or discrimination against non-participation.

Section 4.04 - Yarning Circles and Indigenous Cultural Safety

- Protocols for engagement, assessment, intervention, and review MUST:
 - Centre Aboriginal and Torres Strait Islander leadership and ways of knowing^[14];
 - Include independent Indigenous advisors in all review and oversight functions;
 - Guarantee culturally relevant programs and the right to digital connection with culture and family.
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Division 3 - Oversight, Accountability, and Complaints

Section 4.05 - Designated Oversight Body

- Establish or designate an **Independent Digital Rehabilitation Oversight Committee** (IDROC), which shall:
 - Include representatives from corrections, health, human rights, technology regulation, and Aboriginal and Torres Strait Islander leadership (where applicable);
 - Monitor all deployments, receive reports of adverse events, review implementation data, and publish annual public reports;
 - Supervise compliance with clinical and human rights safeguards.

Section 4.06 - Internal and External Review

- Statutory powers of inspection, investigation, and enforcement reside with both:
 - The IDROC and equivalent prison/mental health inspectors general;
 - Recourse to external Human Rights Commissions, Ombudsman, or Judicial oversight in cases of abuse, misapplication or rights violations.

Section 4.07 - Adverse Event Reporting and Redress

- Mandatory reporting of all:

- Device malfunction or failure;
- Adverse medical events;
- Breaches of privacy or complaints of coercion.
- Clear protocols for notification, investigation, remediation, and compensation, including (but not limited to) recourse to regulatory authorities, courts, and the Australian Human Rights Commission.

Part V - Monitoring, Evaluation, and Research

Division 1 - Research Governance

Section 5.01 - Ethics Review

- All research involving DRT/CA in involuntary settings requires:
 - Review and approval by fully constituted Human Research Ethics Committees, with mandatory inclusion of prisoner representatives, Indigenous advisors, and justice/mental health advocates.

Section 5.02 - Evaluation and Evidence-Based Practice

- Passage of this Act triggers a rolling program of independent evaluation, including:
 - Outcome measurement (recidivism, psychological wellbeing, social reintegration);
 - Privacy and rights impact assessments;
 - User-experience and cultural acceptability studies;
 - Reporting to Parliament biannually.

Part VI - Schedules

Schedule 1 - Amendments to Correctional and Health Legislation

Statute	Amendment Summary
Crimes (Administration of Sentences) Act	New section authorizing use of DRT/CA under clinical oversight, rights-based consent; prohibition on use for punishment; statutory evaluation and audit requirements.
State/Territory Mental Health Acts	Inclusion of DRT/CA under “special treatments” subject to tribunal review; harmonised consent protocols; reporting to National Mental Health Commission; cultural safety in every assessment.

Therapeutic Goods Act and Medical Device Regs	Compulsory registration of implantable and digital health devices used in involuntary settings; extension of ARTG and cybersecurity provisions to correctional and psychiatric settings; mandatory manufacturer data-sharing for oversight.
Privacy Act 1988 (Cth) & State Privacy Acts	Recognition of special status of biometric/neural/rehabilitative data; right to access, correction, and deletion; fast-track complaint procedures for involuntary subjects.
Guardianship Acts	Clarification of substituted decision-making hierarchy, best interests/substituted judgement tests, and right of review for DRT/CA decisions.
Evidence Act (Cth & State)	Protection of therapeutic data and privacy in court/tribunal proceedings.

Schedule 2 - Model Protocols for Implementation

Detailed rules for operational deployment, risk assessment, complaints, and research, including:

- Stepwise clinical protocols for eligibility assessment, including digital inclusion/exclusion criteria and digital literacy evaluation;
 - Templates for information disclosure and consent for DRT/CA interventions;
 - Mandatory language and cultural interpretation services;
 - Procedures for adverse event and breach reporting;
 - Guidelines for regular technology safety audits, including pen-testing and privacy reviews;
 - Ethical use of AI and algorithmic decision-making (drawing on the Council of Europe 2024 AI Prisons Recommendation)^[23].
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Schedule 3 - International Best-Practice Integration

Country/Instrument	Key Safeguards/Features	Points for Australian Alignment
United Nations (Nelson Mandela Rules)	Informed consent, no use for punishment, independent healthcare decisions	Legal binding through Act and statutory guidelines
UN Convention on Rights of Persons with Disabilities	Non-discrimination, respect for integrity, freedom from non-consensual intervention	Direct import into domestic law & anti-discrimination
Europe (Council of Europe AI Prisons Rec. 2024)	Proportionality, legality, human-centered AI, right to human review	Incorporate explicit AI oversight and transparency clauses

US (FDA Medical Device Regulation)	Cybersecurity, adverse event, premarket/postmarket monitoring	Adopt via TGA and ARTG, plus mandatory cybersecurity audit
UK (NICE, NHS Digital Health)	Interoperability, equity, accessibility, patient safety reviews	Require independent evidence-based guideline referencing
South Korea (BCI Device Classifications)	Regulatory class for non-invasive/invasive BCI/prosthetics, post-market surveillance	TGA to mirror BCI risk-tiering for all neural devices

Commentary, Rationale, and Analytical Discussion

1. The Challenge and Promise of Digital Rehabilitation for Involuntary Populations

Digital rehabilitation covers a spectrum of technologies - including tablets, virtual reality (VR), telemedicine, AI-driven risk assessment, and brain-computer interfaces (BCI) - all aimed at reducing recidivism and facilitating social, occupational, and psychological reintegration^[2]. International evidence shows digital tools can support behaviour change, literacy, pro-social identity construction, and access to family and services^{[19][24]}. However, such technologies raise deep human rights and ethical concerns, especially in involuntary and coercive settings. The risks encompass privacy breaches, new forms of surveillance, digital exclusion, coercion, and “technological vulnerability” for marginalized people^[21]. Thus, the legislative framework must be **paradigmatically rights-based, procedurally robust, and operationally feasible**.

2. Statutory Integration and Harmonization

Current Australian frameworks: NSW and Victoria have pioneered the largest-scale deployments of digital rehabilitation in corrections, with tablet devices and platforms supporting education, therapy, and family contact^{[3][25]}. State and territory Mental Health Acts, corrections statutes, and the Therapeutic Goods Act each regulate specific sectors, but their intersection in involuntary digital or prosthetic therapies is incomplete and often inconsistent^[17]. **International benchmarks:** The **Nelson Mandela Rules** are the global minimum for prisoner treatment, with explicit prohibitions on non-consensual treatment, requirements of dignity, informed consent, medical independence, and non-discrimination^[1]. The **CRPD** and European guidelines on medical device cybersecurity and AI governance demand further harmonization around autonomy, privacy, and risk review^{[23][26]}. The statutory amendments and integrations outlined above are thus anchored in, and extend, **global best practices**.

3. Consent and Supported Decision-Making

Consent for digital and cybernetic rehabilitation is **not analogous to ordinary health care consent**. Considerations are compounded by:

- Capacity impairments in mental illness;
- Institutional power imbalances;
- Risks of coercion (subtle or overt);
- Difficulties in achieving “voluntariness” in involuntary settings^[7].

Supported decision-making protocols:

- Prioritise supported over substitute decision-making whenever possible;
- Ensure information is provided in accessible, culturally safe formats;
- Protect the right to refuse or withdraw at any time, with periodic review;
- Require external, multidisciplinary, and culturally competent oversight for high-risk or irreversible interventions.

4. Medical Device and Technology Oversight

The proliferation of BCIs, neuroprosthetics, and AI-driven applications means a **robust and adaptable regulatory architecture** must cover both established and emerging devices.

- **TGA and ARTG registration** is mandatory for any implantable, wearable, or digital device used as a therapeutic or augmentation tool. New guidelines for **digital mental health devices** provide specific exclusion and inclusion criteria, with software that “guides diagnosis or treatment” subject to more intensive review^{[27][16]}.
- **Cybersecurity** and data integrity obligations are integral: as neural and biometric data become both medically and forensically significant, the potential for “mental privacy” breaches must be anticipated and regulated at the highest level^[28].
- **International equivalence:** Standards such as ISO 14971 (risk management), IEC 62304 (software lifecycle), and GDPR-equivalent privacy laws are crucial for cross-border and multinational technology governance.

Table: Regulatory Requirements Matrix

Device	Registration	Consent Level	Oversight	Reporting	Privacy
Tablet (Education/Therapy)	ARTG/Software-DeviceReg	Supported/Voluntary	Correctional/Health	Adverse event	Privacy Act; Data minimization
BCI Implant	ARTG + TGA	Substitute w/ Tribunal	Ethics/HR Committee	All incidents	Privacy Act; High alert
VR Therapy	ARTG (if medical)	Supported/Voluntary	Clinical Lead	Usage, adverse	Privacy Act

Wearable Prosthetic	ARTG	Supported/Voluntary	Clinical Lead	Maintenance	Privacy Act
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Each device deployment must be tailored with reference to clinical need, human rights, risk analysis, and cultural context.

5. Human Rights, Dignity, and Inclusion

A digital rehabilitation framework is only justifiable if it **enhances, rather than replaces, human dignity and capacity**. The Mandela Rules articulate:

- No technological or scientific advancement may be used as a pretext for discrimination, coercion, or adverse classification;
- All interventions must be subject to regular, independent, and ideally external scrutiny;
- Special care for Indigenous and vulnerable groups is not only a legal but a moral necessity, with protocols for co-designed and Indigenous-informed digital and restorative practices^[15].

6. Digital Inclusion and Equity

The risk of a new “tech divide” in prisons is clear: those with low digital literacy or fewer financial means may be excluded from rehabilitative opportunities, further exacerbating disadvantage^{[19][4]}. Equitable access mandates:

- Universal access and inclusion-by-design;
- Culturally relevant interfaces and training opportunities for digital tools;
- Avoidance of “paywall” models or predatory commercial arrangements;
- Transparent user-fee policy for extra services with public benefit justifications.

7. Oversight and Accountability

Legislative language mandates oversight that is **independent, resourced, and multi-disciplinary**, blending justice, health, digital, legal, and Indigenous expertise. Best practices include:

- Statutory powers of entry, inspection, information, and publication for the IDROC (or equivalent);
- Regular, public reporting and community consultation;
- Parallel complaint mechanisms via Human Rights Commissions;
- International peer learning and adoption of robust models for inspection (e.g., Scandinavian ombuds systems)^[30].

Concluding Section - Legislative Implementation, Future Directions, and Periodic Review

- The Act shall come into effect twelve months from Royal Assent, with a six-month framework for systems development, training, and preliminary independent evaluation.
 - A statutory review committee will be established at the five-year mark, tasked with reassessing technological, ethical, and rights-based safeguards, with the power to propose amendments.
 - All subordinate policy documents, regulations, and operational directives issued pursuant to this Act must be published online and subject to public consultation.
 - Australia's leadership on digital and cybernetic rehabilitation in involuntary settings will be measured by its ability to **balance innovation, justice, human dignity, and social inclusion** - with ongoing attention to the perspectives of those most affected, especially Indigenous peoples, people with disabilities, and survivors of trauma.
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Schedules

Schedule 1: Definitions and Interpretative Provisions

- See **Definitions** and **Eligibility** sections above.

Schedule 2: Model Policy and Practice Guidelines

- Templates for consent, risk assessment, adverse events, and staff/peer-led digital literacy programs.
- Engagement protocols for Aboriginal and Torres Strait Islander and other multiply marginalized communities.

Schedule 3: Review and Evaluation Timeline

- Year 1: Baseline audit
- Year 3: Preliminary evaluation, public consultation
- Year 5: Full parliamentary review

This legislative proposal provides Australia and partner jurisdictions with a comprehensive, internationally aligned, evidence-based, and ethically anchored model for governing digital rehabilitation and cybernetic augmentation in involuntary corrections and mental health settings, protecting the rights and dignity of some of the most vulnerable people while enabling therapeutic innovation and societal safety.

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